

tion of the mandate to the objecting parties violated RFRA, we left it to the Departments to decide how best to rectify this problem. See 573 U.S., at 736; 79 Fed. Reg. 51118 (2014) (proposing to modify the accommodation to extend it to closely held corporations in light of *Hobby Lobby*); 80 Fed. Reg. 41324 (final rule explaining that “[t]he Departments believe that the definition adopted in these regulations complies with and goes beyond what is required by RFRA and *Hobby Lobby*”).

The same principle applies here. Once it is recognized that the prior accommodation violated RFRA in some of its applications, it was incumbent on the Departments to eliminate those violations, and they had discretion in crafting what they regarded as the best solution.

The solution they devised cures the problem, and it is not clear that any narrower exemption would have been sufficient with respect to parties with religious objections to the accommodation. As noted, after great effort, the Government concluded that it was not possible to solve the problem without using an “employer’s plan, issuer, or third party administrator.” 83 Fed. Reg. 57546. As a result, the Departments turned to the current rule, under which an objecting party must certify that it “objects, based on its sincerely held religious beliefs, to its establishing, maintaining, providing, offering, or arranging for (as applicable)” either “[c]overage or payments for some or all contraceptive services” or “[a] plan, issuer, or third party administrator that provides or arranges such coverage or payments.” 45 CFR §§ 147.132(a)(2)(i)–(ii).

The States take exception to the new religious rule on several grounds. First, they complain that it grants an exemption to some employers who were satisfied with the prior accommodation, but there is little basis for this argument. An employer who is satisfied with the accommodation may continue to operate under that regime. See §§ 147.131(c)–(d); 83 Fed. Reg. 57569–57571. And unless an employer has a religious objection to the accommodation, it is unclear why

ALITO, J., concurring

an employer would give it up. The accommodation does not impose any cost on an employer, and it provides an added benefit for the employer's work force.

The States also object to the new rule because it makes exemptions available to publicly traded corporations, but the Government is "not aware" of any publicly traded corporations that object to compliance with the mandate. *Id.*, at 57562. For all practical purposes, therefore, it is not clear that the new rule's provisions concerning entities that object to the mandate on religious grounds go any further than necessary to bring the mandate into compliance with RFRA.

In any event, while RFRA requires the Government to employ the least restrictive means of furthering a compelling interest that burdens religious belief, it does not require the converse—that an accommodation of religious belief be narrowly tailored to further a compelling interest. The latter approach, which is advocated by the States, gets RFRA entirely backwards. See Brief for Respondents 45 ("RFRA could require the religious exemption only if it was the least restrictive means of furthering [the Government's compelling interest]"). Nothing in RFRA requires that a violation be remedied by the narrowest permissible corrective.

Needless to say, the remedy for a RFRA problem cannot violate the Constitution, but the new rule does not have that effect. The Court has held that there is a constitutional right to purchase and use contraceptives. *Griswold v. Connecticut*, 381 U. S. 479 (1965); *Carey v. Population Services Int'l*, 431 U. S. 678 (1977). But the Court has never held that there is a constitutional right to free contraceptives.

The dissent and the court below suggest that the new rule is improper because it imposes burdens on the employees of entities that the rule exempts, see *post*, at 723–726; 930 F. 3d, at 573–574,¹³ but the rule imposes no such burden. A

¹³ Both the dissent and the court below refer to the statement in *Cutter v. Wilkinson*, 544 U. S. 709, 720 (2005), that "courts must take adequate account of the burdens a requested accommodation may impose on nonben-

woman who does not have the benefit of contraceptive coverage under her employer's plan is not the victim of a burden imposed by the rule or her employer. She is simply not the beneficiary of something that federal law does not provide. She is in the same position as a woman who does not work outside the home or a woman whose health insurance is provided by a grandfathered plan that does not pay for contraceptives or a woman who works for a small business that may not provide any health insurance at all.

* * *

I would hold not only that it was appropriate for the Departments to consider RFRA but also that the Departments were required by RFRA to create the religious exemption (or something very close to it). I would bring the Little Sisters' legal odyssey to an end.

JUSTICE KAGAN, with whom JUSTICE BREYER joins, concurring in the judgment.

I would uphold HRSA's statutory authority to exempt certain employers from the contraceptive-coverage mandate, but for different reasons than the Court gives. I also write separately because I question whether the exemptions can survive administrative law's demand for reasoned decision-making. That issue remains open for the lower courts to address.

The majority and dissent dispute the breadth of the delegation in the Women's Health Amendment to the ACA. The

eficiaries," but that statement was made in response to the argument that RFRA's twin, the Religious Land Use and Institutionalized Persons Act, 42 U.S.C. § 2000cc *et seq.*, violated the Establishment Clause. The only case cited by *Cutter* in connection with this statement, *Estate of Thornton v. Caldor, Inc.*, 472 U.S. 703 (1985), involved a religious accommodation that the Court held violated the Establishment Clause. Before this Court, the States do not argue—and there is no basis for an argument—that the new rule violates that Clause.

KAGAN, J., concurring in judgment

Amendment states that a health plan or insurer must offer coverage for “preventive care and screenings . . . as provided for in comprehensive guidelines supported by [HRSA] for purposes of this paragraph.” 42 U. S. C. § 300gg–13(a)(4). The disputed question is just what HRSA can “provide for.” Both the majority and the dissent agree that HRSA’s guidelines can differentiate among preventive services, mandating coverage of some but not others. The opinions disagree about whether those guidelines can also differentiate among health plans, exempting some but not others from the contraceptive-coverage requirement. On that question, all the two opinions have in common is equal certainty they are right. Compare *ante*, at 677 (majority opinion) (Congress “enacted expansive language offer[ing] no indication whatever that the statute limits what HRSA can designate as preventive care and screenings or who must provide that coverage” (internal quotation marks omitted)), with *post*, at 718 (GINSBURG, J., dissenting) (“Nothing in [the statute] accord[s] HRSA authority” to decide “*who* must provide coverage” (internal quotation marks omitted; emphasis in original)).

Try as I might, I do not find that kind of clarity in the statute. Sometimes when I squint, I read the law as giving HRSA discretion over all coverage issues: The agency gets to decide who needs to provide what services to women. At other times, I see the statute as putting the agency in charge of only the “what” question, and not the “who.” If I had to, I would of course decide which is the marginally better reading. But *Chevron* deference was built for cases like these. See *Chevron U. S. A. Inc. v. Natural Resources Defense Council, Inc.*, 467 U. S. 837, 842–843 (1984); see also *Arlington v. FCC*, 569 U. S. 290, 301 (2013) (holding that *Chevron* applies to questions about the scope of an agency’s statutory authority). *Chevron* instructs that a court facing statutory ambiguity should accede to a reasonable interpretation by the implementing agency. The court should do so because

the agency is the more politically accountable actor. See 467 U. S., at 865–866. And it should do so because the agency’s expertise often enables a sounder assessment of which reading best fits the statutory scheme. See *id.*, at 865.

Here, the Departments have adopted the majority’s reading of the statutory delegation ever since its enactment. Over the course of two administrations, the Departments have shifted positions on many questions involving the Women’s Health Amendment and the ACA more broadly. But not on whether the Amendment gives HRSA the ability to create exemptions to the contraceptive-coverage mandate. HRSA adopted the original church exemption on the same capacious understanding of its statutory authority as the Departments endorse today. See 76 Fed. Reg. 46623 (2011) (“In the Departments’ view, it is appropriate that HRSA, in issuing these Guidelines, takes into account the effect on the religious beliefs of certain religious employers if coverage of contraceptive services were required”).¹ While the exemption itself has expanded, the Departments’ reading of the statutory delegation—that the law gives HRSA discretion over the “who” question—has remained the same. I would defer to that longstanding and reasonable interpretation.

¹The First Amendment cannot have separately justified the church exemption, as the dissent suggests. See *post*, at 721–722 (opinion of GINSBURG, J.). That exemption enables a religious institution to decline to provide contraceptive coverage to *all* its employees, from a minister to a building custodian. By contrast, the so-called ministerial exception of the First Amendment (which the dissent cites, see *post*, at 722) extends only to *select* employees, having ministerial status. See *Our Lady of Guadalupe School v. Morrissey-Berru*, 591 U. S. 732, 749–751 (2020); *Hosanna-Tabor Evangelical Lutheran Church and School v. EEOC*, 565 U. S. 171, 190 (2012). (Too, this Court has applied the ministerial exception only to protect religious institutions from employment discrimination suits, expressly reserving whether the exception excuses their noncompliance with other laws. See *id.*, at 196.) And there is no general constitutional immunity, over and above the ministerial exception, that can protect a religious institution from the law’s operation.

KAGAN, J., concurring in judgment

But that does not mean the Departments should prevail when these cases return to the lower courts. The States challenged the exemptions not only as outside HRSA’s statutory authority, but also as “arbitrary [and] capricious.” 5 U.S.C. § 706(2)(A). Because the courts below found for the States on the first question, they declined to reach the second. That issue is now ready for resolution, unaffected by today’s decision. An agency acting within its sphere of delegated authority can of course flunk the test of “reasoned decisionmaking.” *Michigan v. EPA*, 576 U.S. 743, 750 (2015). The agency does so when it has not given “a satisfactory explanation for its action”—when it has failed to draw a “rational connection” between the problem it has identified and the solution it has chosen, or when its thought process reveals “a clear error of judgment.” *Motor Vehicle Mfrs. Assn. of United States, Inc. v. State Farm Mut. Automobile Ins. Co.*, 463 U.S. 29, 43 (1983) (internal quotation marks omitted). Assessed against that standard of reasonableness, the exemptions HRSA and the Departments issued give every appearance of coming up short.²

Most striking is a mismatch between the scope of the religious exemption and the problem the agencies set out to address. In the Departments’ view, the exemption was “necessary to expand the protections” for “certain entities and individuals” with “religious objections” to contraception. 83 Fed. Reg. 57537 (2018). Recall that under the old system, an employer objecting to the contraceptive mandate for religious reasons could avail itself of the “self-certification accommodation.” *Ante*, at 667. Upon making the certification, the employer no longer had “to contract, arrange, [or] pay” for contraceptive coverage; instead, its insurer would bear the services’ cost. 78 Fed. Reg. 39874 (2013). That device dispelled some employers’ objections—but not all. The Lit-

²I speak here only of the substantive validity of the exemptions. I agree with the Court that the final rules issuing the exemptions were procedurally valid.

tle Sisters, among others, maintained that the accommodation itself made them complicit in providing contraception. The measure thus failed to “assuage[]” their “sincere religious objections.” 82 Fed. Reg. 47799 (2017). Given that fact, the Departments might have chosen to exempt the Little Sisters and other still-objecting groups from the mandate. But the Departments went further still. Their rule exempted all employers with objections to the mandate, even if the accommodation met their religious needs. In other words, the Departments exempted employers who had no religious objection to the status quo (because they did not share the Little Sisters’ views about complicity). The rule thus went beyond what the Departments’ justification supported—raising doubts about whether the solution lacks a “rational connection” to the problem described. *State Farm*, 463 U.S., at 43.³

And the rule’s overbreadth causes serious harm, by the Departments’ own lights. In issuing the rule, the Departments chose to retain the contraceptive mandate itself. See 83 Fed. Reg. 57537. Rather than dispute HRSA’s prior finding that the mandate is “necessary for women’s health and well-being,” the Departments left that determination in place. HRSA, Women’s Preventive Services Guidelines

³ At oral argument, the Solicitor General argued that the rule’s overinclusion is harmless because the accommodation remains available to all employers who qualify for the exemption. See Tr. of Oral Arg. 20–23. But in their final rule, the Departments themselves acknowledged the prospect that some employers without a religious objection to the accommodation would switch to the exemption. See 83 Fed. Reg. 57576–57577 (“Of course, some of the[] religious” institutions that “do not conscientiously oppose participating” in the accommodation “may opt for the expanded exemption[,] but others might not”); *id.*, at 57561 (“[I]t is not clear to the Departments” how many of the religious employers who had used the accommodation without objection “will choose to use the expanded exemption instead”). And the Solicitor General, when pressed at argument, could offer no evidence that, since the rule took effect, employers without the Little Sisters’ complicity beliefs had declined to avail themselves of the new exemption. Tr. of Oral Arg. 22.

KAGAN, J., concurring in judgment

(Dec. 2019), www.hrsa.gov/womens-guidelines-2019; see 83 Fed. Reg. 57537. The Departments thus committed themselves to minimizing the impact on contraceptive coverage, even as they sought to protect employers with continuing religious objections. But they failed to fulfill that commitment to women. Remember that the accommodation preserves employees' access to cost-free contraceptive coverage, while the exemption does not. See *ante*, at 666–667. So the Departments (again, according to their own priorities) should have exempted only employers who had religious objections to the accommodation—not those who viewed it as a religiously acceptable device for complying with the mandate. The Departments' contrary decision to extend the exemption to those without any religious need for it yielded all costs and no benefits. Once again, that outcome is hard to see as consistent with reasoned judgment. See *State Farm*, 463 U. S., at 43.⁴

Other aspects of the Departments' handiwork may also prove arbitrary and capricious. For example, the Departments allow even publicly traded corporations to claim a religious exemption. See 83 Fed. Reg. 57562–57563. That option is unusual enough to raise a serious question about whether the Departments adequately supported their choice. Cf. *Burwell v. Hobby Lobby Stores, Inc.*, 573 U. S. 682, 717 (2014) (noting the oddity of “a publicly traded corporation asserting RFRA rights”). Similarly, the Departments offer an exemption to employers who have moral, rather than religious, objections to the contraceptive mandate. Perhaps

⁴ In a brief passage in the interim final rule, the Departments suggested that an exemption is “more workable” than the accommodation in addressing religious objections to the mandate. 82 Fed. Reg. 47806. But the Departments continue to provide the accommodation to any religious employers who request that option, thus maintaining a two-track system. See *ante*, at 671; n. 3, *supra*. So ease of administration cannot support, at least without more explanation, the Departments' decision to offer the exemption more broadly than needed.

there are sufficient reasons for that decision—for example, a desire to stay neutral between religion and non-religion. See 83 Fed. Reg. 57603–57604. But RFRA cast a long shadow over the Departments’ rulemaking, see *ante*, at 680–683, and that statute does not apply to those with only moral scruples. So a careful agency would have weighed anew, in this different context, the benefits of exempting more employers from the mandate against the harms of depriving more women of contraceptive coverage. In the absence of such a reassessment, it seems a close call whether the moral exemption can survive.

None of this is to say that the Departments could not issue a valid rule expanding exemptions from the contraceptive mandate. As noted earlier, I would defer to the Departments’ view of the scope of Congress’s delegation. See *supra*, at 706. That means the Departments (assuming they act hand-in-hand with HRSA) have wide latitude over exemptions, so long as they satisfy the requirements of reasoned decisionmaking. But that “so long as” is hardly nothing. Even in an area of broad statutory authority—maybe especially there—agencies must rationally account for their judgments.

JUSTICE GINSBURG, with whom JUSTICE SOTOMAYOR joins, dissenting.

In accommodating claims of religious freedom, this Court has taken a balanced approach, one that does not allow the religious beliefs of some to overwhelm the rights and interests of others who do not share those beliefs. See, *e. g.*, *Estate of Thornton v. Caldor, Inc.*, 472 U.S. 703, 708–710 (1985); *United States v. Lee*, 455 U.S. 252, 258–260 (1982). Today, for the first time, the Court casts totally aside countervailing rights and interests in its zeal to secure religious rights to the *n*th degree. Specifically, in the Women’s Health Amendment to the Patient Protection and Affordable

GINSBURG, J., dissenting

Care Act (ACA), 124 Stat. 119; 155 Cong. Rec. 28841 (2009), Congress undertook to afford gainfully employed women comprehensive, seamless, no-cost insurance coverage for preventive care protective of their health and well-being. Congress delegated to a particular agency, the Health Resources and Services Administration (HRSA), authority to designate the preventive-care insurance should cover. HRSA included in its designation all contraceptives approved by the Food and Drug Administration (FDA).

Destructive of the Women's Health Amendment, this Court leaves women workers to fend for themselves, to seek contraceptive coverage from sources other than their employer's insurer, and, absent another available source of funding, to pay for contraceptive services out of their own pockets. The Constitution's Free Exercise Clause, all agree, does not call for that imbalanced result.¹ Nor does the Religious Freedom Restoration Act of 1993 (RFRA), 42 U. S. C. § 2000bb *et seq.*, condone harm to third parties occasioned by entire disregard of their needs. I therefore dissent from the Court's judgment, under which, as the Government estimates, between 70,500 and 126,400 women would immediately lose access to no-cost contraceptive services. On the merits, I would affirm the judgment of the U. S. Court of Appeals for the Third Circuit.

¹ In *Employment Div., Dept. of Human Resources of Ore. v. Smith*, 494 U. S. 872 (1990), the Court explained that "the right of free exercise does not relieve an individual of the obligation to comply with a valid and neutral law of general applicability on the ground that the law proscribes (or prescribes) conduct that his religion prescribes (or proscribes)." *Id.*, at 879 (internal quotation marks omitted). The requirement that insurers cover FDA-approved methods of contraception "applies generally, . . . trains on women's well-being, not on the exercise of religion, and any effect it has on such exercise is incidental." *Burwell v. Hobby Lobby Stores, Inc.*, 573 U. S. 682, 745 (2014) (GINSBURG, J., dissenting). *Smith* forecloses "[a]ny First Amendment Free Exercise Clause claim [one] might assert" in opposition to that requirement. 573 U. S., at 744.

I

A

Under the ACA, an employer-sponsored “group health plan” must cover specified “preventive health services” without “cost sharing,” 42 U.S.C. § 300gg–13, *i. e.*, without such out-of-pocket costs as copays or deductibles.² Those enumerated services did not, in the original draft bill, include preventive care specific to women. “To correct this oversight, Senator Barbara Mikulski introduced the Women’s Health Amendment,” now codified at § 300gg–13(a)(4). *Burwell v. Hobby Lobby Stores, Inc.*, 573 U.S. 682, 741 (2014) (GINSBURG, J., dissenting); see also 155 Cong. Rec. 28841. This provision was designed “to promote equality in women’s access to health care,” countering gender-based discrimination and disparities in such access. Brief for 186 Members of the United States Congress as *Amici Curiae* 6 (hereinafter Brief for 186 Members of Congress). Its proponents noted, *inter alia*, that “[w]omen paid significantly more than men for preventive care,” and that “cost barriers operated to block many women from obtaining needed care at all.” *Hobby Lobby*, 573 U.S., at 742 (GINSBURG, J., dissenting); see, *e. g.*, 155 Cong. Rec. 28844 (statement of Sen. Hagan) (“When . . . women had to choose between feeding their children, paying the rent, and meeting other financial obligations, they skipped important preven-

²This requirement does not apply to employers with fewer than 50 employees, 26 U.S.C. § 4980H(c)(2), or “grandfathered health plans”—plans in existence on March 23, 2010 that have not thereafter made specified changes in coverage, 42 U.S.C. § 18011(a), (e); 45 CFR § 147.140(g) (2018). “Federal statutes often include exemptions for small employers, and such provisions have never been held to undermine the interests served by these statutes.” *Hobby Lobby*, 573 U.S., at 763 (GINSBURG, J., dissenting). “[T]he grandfathering provision,” “far from ranking as a categorical exemption, . . . is temporary, intended to be a means for gradually transitioning employers into mandatory coverage.” *Id.*, at 764 (internal quotation marks omitted).

GINSBURG, J., dissenting

tive screenings and took a chance with their personal health.”).

Due to the Women’s Health Amendment, the preventive health services that group health plans must cover include, “with respect to women,” “preventive care and screenings . . . provided for in comprehensive guidelines supported by [HRSA].” § 300gg–13(a)(4). Pursuant to this instruction, HRSA undertook, after consulting the Institute of Medicine,³ to state “what preventive services are necessary for women’s health and well-being and therefore should be considered in the development of comprehensive guidelines for preventive services for women.”⁴ The resulting “Women’s Preventive Services Guidelines” issued in August 2011.⁵ Under these guidelines, millions of women who previously had no, or poor quality, health insurance gained cost-free access, not only to contraceptive services but as well to, *inter alia*, annual checkups and screenings for breast cancer, cervical cancer, postpartum depression, and gestational diabetes.⁶ As to contraceptive services, HRSA directed that, to implement § 300gg–13(a)(4), women’s preventive services encompass “all [FDA]-approved contraceptive methods, sterilization procedures, and patient education and counseling for all women with reproductive capacity.”⁷

³ “The [Institute of Medicine] is an arm of the National Academy of Sciences, an organization Congress established for the explicit purpose of furnishing advice to the Government.” *Id.*, at 742, n. 3 (internal quotation marks omitted).

⁴ HRSA, U.S. Dept. of Health and Human Services (HHS), Women’s Preventive Services Guidelines, www.hrsa.gov/womens-guidelines/index.html.

⁵ 77 Fed. Reg. 8725 (2012).

⁶ HRSA, HHS, Women’s Preventive Services Guidelines, *supra*.

⁷ 77 Fed. Reg. 8725 (alterations and internal quotation marks omitted). Proponents of the Women’s Health Amendment specifically anticipated that HRSA would require coverage of family planning services. See, e.g., 155 Cong. Rec. 28841 (2009) (statement of Sen. Boxer); *id.*, at 28843 (statement of Sen. Gillibrand); *id.*, at 28844 (statement of Sen. Mikulski); *id.*, at 28869 (statement of Sen. Franken); *id.*, at 28876 (statement of Sen. Cardin);

Ready access to contraceptives and other preventive measures for which Congress set the stage in § 300gg-13(a)(4) both safeguards women's health and enables women to chart their own life's course. Effective contraception, it bears particular emphasis, "improves health outcomes for women and [their] children," as "women with unintended pregnancies are more likely to receive delayed or no prenatal care" than women with planned pregnancies. Brief for 186 Members of Congress 5 (internal quotation marks omitted); Brief for American College of Obstetricians and Gynecologists et al. as *Amici Curiae* 10 (hereinafter ACOG Brief) (similar). Contraception is also "critical for individuals with underlying medical conditions that would be further complicated by pregnancy" "has . . . health benefits unrelated to preventing pregnancy," (*e. g.*, it can reduce the risk of endometrial and ovarian cancer), Brief for National Women's Law Center et al. as *Amici Curiae* 23-24, 26 (hereinafter NWLC Brief), and "improves women's social and economic status," by "allow[ing] [them] to invest in higher education and a career with far less risk of an unplanned pregnancy," Brief for 186 Members of Congress 5-6 (internal quotation marks omitted).

B

For six years, the Government took care to protect women employees' access to critical preventive health services while accommodating the diversity of religious opinion on contraception. The Internal Revenue Service (IRS), the Employee Benefits Security Administration (EBSA), and the Center for Medicare and Medicaid Services (CMS) crafted a narrow exemption relieving houses of worship, "their integrated auxiliaries," "conventions or associations of churches," and "religious order[s]" from the contraceptive-coverage requirement. 76 Fed. Reg. 46623 (2011). For

ibid. (statement of Sen. Feinstein); *id.*, at 29307 (statement of Sen. Murray).

GINSBURG, J., dissenting

other nonprofit and closely held for-profit organizations opposed to contraception on religious grounds, the agencies made available an accommodation rather than an exemption. See 78 Fed. Reg. 39874 (2013); *Hobby Lobby*, 573 U. S., at 730–731.

“Under th[e] accommodation, [an employer] can self-certify that it opposes providing coverage for particular contraceptive services. See 45 CFR §§ 147.131(b)(4), (c)(1) [(2013)]; 26 CFR §§ 54.9815–2713A(a)(4), (b). If [an employer] makes such a certification, the [employer’s] insurance issuer or third-party administrator must ‘[e]xpressly exclude contraceptive coverage from the group health insurance coverage provided in connection with the group health plan’ and ‘[p]rovide separate payments for any contraceptive services required to be covered’ without imposing ‘any cost-sharing requirements . . . on the [employer], the group health plan, or plan participants or beneficiaries.’ 45 CFR § 147.131(c)(2); 26 CFR § 54.9815–2713A(c)(2).” *Id.*, at 731 (some alterations in original).⁸

The self-certification accommodation, the Court observed in *Hobby Lobby*, “does not impinge on [an employer’s] belief that providing insurance coverage for . . . contraceptives . . . violates [its] religion.” *Ibid.* It serves “a Government interest of the highest order,” *i. e.*, providing women employees “with cost-free access to all FDA-approved methods of contraception.” *Id.*, at 729. And “it serves [that] stated interest[t] . . . well.” *Id.*, at 731; see *id.*, at 693 (Government properly accommodated employer’s religion-based objection

⁸This opinion refers to the contraceptive-coverage accommodation made in 2013 as the “self-certification accommodation.” See *ante*, at 667 (opinion of the Court). Although this arrangement “requires the issuer to bear the cost of [contraceptive] services, HHS has determined that th[e] obligation will not impose any net expense on issuers because its cost will be less than or equal to the cost savings resulting from th[ose] services.” *Hobby Lobby*, 573 U. S., at 698–699.

to covering contraceptives under employer’s health insurance plan when the harm to women of doing so “would be precisely zero”). Since the ACA’s passage, “[gainfully employed] [w]omen, particularly in lower-income groups, have reported greater affordability of coverage, access to health care, and receipt of preventive services.” Brief for 186 Members of Congress 21.

C

Religious employers, including petitioner Little Sisters of the Poor Saints Peter and Paul Home (Little Sisters), nonetheless urge that the self-certification accommodation renders them “complicit in providing [contraceptive] coverage to which they sincerely object.” Brief for Little Sisters 35. In 2017, responsive to the pleas of such employers, the Government abandoned its effort to both end discrimination against employed women in access to preventive services and accommodate religious exercise. Under new rules drafted not by HRSA, but by the IRS, EBSA, and CMS, *any* “non-governmental employer”—even a publicly traded for-profit company—can avail itself of the religious exemption previously reserved for houses of worship. 82 Fed. Reg. 47792 (2017) (interim final rule); 45 CFR § 147.132(a)(1)(i)(E) (2018).⁹ More than 2.9 million Americans—including approximately 580,000 women of childbearing age—receive insurance through organizations newly eligible for this blanket exemption. 83 Fed. Reg. 57577–57578 (2018). Of cardinal significance, the exemption contains no alternative mechanism to ensure affected women’s continued access to contraceptive coverage. See 45 CFR § 147.132.

Pennsylvania and New Jersey, respondents here, sued to enjoin the exemption. Their lawsuit posed this core ques-

⁹ Nonprofit and closely held for-profit organizations with “sincerely held moral convictions” against contraception also qualify for the exemption. 45 CFR § 147.133(a)(1)(i), (a)(2). Unless otherwise noted, this opinion refers to the religious and moral exemptions together as “the exemption” or “the blanket exemption.”

GINSBURG, J., dissenting

tion: May the Government jettison an arrangement that promotes women workers' well-being while accommodating employers' religious tenets and, instead, defer entirely to employers' religious beliefs, although that course harms women who do not share those beliefs? The District Court answered "no," and preliminarily enjoined the blanket exemption nationwide. 281 F. Supp. 3d 553, 585 (ED Pa. 2017). The Court of Appeals affirmed. 930 F. 3d 543, 576 (CA3 2019). The same question is now presented for ultimate decision by this Court.

II

Despite Congress' endeavor, in the Women's Health Amendment to the ACA, to redress discrimination against women in the provision of healthcare, the exemption the Court today approves would leave many employed women just where they were before insurance issuers were obliged to cover preventive services for them, cost free. The Government urges that the ACA itself authorizes this result, by delegating to HRSA authority to exempt employers from the contraceptive-coverage requirement. This argument gains the Court's approbation. It should not.

A

I begin with the statute's text. But see *ante*, at 678 (opinion of the Court) (overlooking my starting place). The ACA's preventive-care provision, 42 U.S.C. § 300gg-13(a), reads in full:

"A group health plan and a health insurance issuer offering group or individual health insurance coverage shall, at a minimum provide coverage for and shall not impose any cost sharing requirements for—

"(1) evidence-based items or services that have in effect a rating of 'A' or 'B' in the current recommendations of the United States Preventive Services Task Force;

"(2) immunizations that have in effect a recommendation from the Advisory Committee on Immunization

Practices of the Centers for Disease Control and Prevention with respect to the individual involved; . . .

“(3) with respect to infants, children, and adolescents, evidence-informed preventive care and screenings provided for in the comprehensive guidelines supported by [HRSA; and]

“(4) with respect to women, such additional preventive care and screenings not described in paragraph (1) as provided for in comprehensive guidelines supported by [HRSA] for purposes of this paragraph.”

At the start of this provision, Congress instructed who is to “provide coverage for” the specified preventive health services: “group health plan[s]” and “health insurance issuer[s].” §300gg-13(a). As the Court of Appeals explained, paragraph (a)(4), added by the Women’s Health Amendment, granted HRSA “authority to issue ‘comprehensive guidelines’ concern[ing] the *type* of services” group health plans and health insurance issuers must cover with respect to women. 930 F. 3d, at 570 (emphasis added). Nothing in paragraph (a)(4) accorded HRSA “authority to undermine Congress’s [initial] directive,” stated in subsection (a), “concerning *who* must provide coverage for these services.” *Ibid.* (emphasis added).

The Government argues otherwise, asserting that “[t]he sweeping authorization for HRSA to ‘provide[] for’ and ‘support[]’ guidelines ‘for purposes of’ the women’s preventive-services mandate clearly grants HRSA the power not just to specify what services should be covered, but also to provide appropriate exemptions.” Brief for HHS et al. 15.¹⁰ This terse statement—the entirety of the Government’s textual case—slights the language Congress employed. Most visibly, the Government does not endeavor to explain how any

¹⁰ This opinion uses “Brief for HHS et al.” to refer to the Brief for Petitioners in No. 19–454, filed on behalf of the Departments of HHS, Treasury, and Labor, the Secretaries of those Departments, and the President.

GINSBURG, J., dissenting

language in paragraph (a)(4) counteracts Congress' opening instruction in § 300gg–13(a) that group health plans “shall . . . provide” specified services. See *supra*, at 717–718.

The Court embraces, and the opinion concurring in the judgment adopts, the Government's argument. The Court correctly acknowledges that HRSA has broad discretion to determine *what* preventive services insurers should provide for women. *Ante*, at 678. But it restates that HRSA's “discretion [is] equally unchecked in other areas, including the ability to identify and create exemptions from its own Guidelines.” *Ibid.* See also *ante*, at 705–706 (KAGAN, J., concurring in judgment) (agreeing with this interpretation). Like the Government, the Court and the opinion concurring in the judgment shut from sight § 300gg–13(a)'s overarching direction that group health plans and health insurance issuers “shall” cover the specified services. See *supra*, at 717–718 and this page. That “‘absent provision[s] cannot be supplied by the courts,’” *ante*, at 677 (quoting *Rotkiske v. Klemm*, 589 U. S. 8, 14 (2019), militates *against* the Court's conclusion, not in favor of it. Where Congress wanted to exempt certain employers from the ACA's requirements, it said so expressly. See, e. g., *supra*, at 712, n. 2. Section 300gg–13(a)(4) includes no such exemption. See *supra*, at 717–718 and this page.¹¹

B

The position advocated by the Government and endorsed by the Court and the opinion concurring in the judgment encounters further obstacles.

Most saliently, the language in § 300gg–13(a)(4) mirrors that in § 300gg–13(a)(3), the provision addressing *children's* preventive health services. Not contesting here that HRSA

¹¹ The only language to which the Court points in support of its contrary conclusion is the phrase “as provided for.” See *ante*, at 676. This phrase modifies “additional preventive care and screenings.” § 300gg–13(a)(4). It therefore speaks to *what* services shall be provided, not *who* must provide them.

lacks authority to exempt group health plans from the children’s preventive-care guidelines, the Government attempts to distinguish paragraph (a)(3) from paragraph (a)(4). Brief for HHS et al. 16–17. The attempt does not withstand inspection.

The Government first observes that (a)(4), unlike (a)(3), contemplates guidelines created “*for purposes of this paragraph.*” (Emphasis added.) This language does not speak to the scope of the guidelines HRSA is charged to create. Moreover, the Government itself accounts for this textual difference: The children’s preventive-care guidelines described in paragraph (a)(3) were “preexisting guidelines . . . developed for purposes unrelated to the ACA.” Brief for HHS et al. 16. The guidelines on women’s preventive care, by contrast, did not exist before the ACA; they had to be created “for purposes of” the preventive-care mandate. § 300gg–13(a)(4). The Government next points to the modifier “evidence-informed” placed in (a)(3), but absent in (a)(4). This omission, however it may bear on the kind of preventive services for women HRSA can require group health insurance to cover, does not touch or concern *who* is required to cover those services.¹²

HRSA’s role within HHS also tugs against the Government’s, the Court’s, and the opinion concurring in the judgment’s construction of § 300gg–13(a)(4). That agency was a logical choice to determine *what* women’s preventive services should be covered, as its mission is to “improve health care access” and “eliminate health disparities.”¹³ First and foremost, § 300gg–13(a)(4) is directed at eradicating gender-based disparities in access to preventive care. See *supra*, at 712–713. Overlooked by the Court, see *ante*, at 675–679, and the

¹² The Court does not say whether, in its view, the exemption authority it claims for women’s preventive care exists as well for HRSA’s children’s preventive-care guidelines.

¹³ HRSA, HHS, Organization, www.hrsa.gov/about/organization/index.html.

GINSBURG, J., dissenting

opinion concurring in the judgment, see *ante*, at 705–706 (opinion of KAGAN, J.), HRSA’s expertise does not include any proficiency in delineating religious and moral exemptions. One would not, therefore, expect Congress to delegate to HRSA the task of crafting such exemptions. See *King v. Burwell*, 576 U. S. 473, 486 (2015) (“It is especially unlikely that Congress would have delegated this decision to [an agency] which has no expertise in . . . policy of this sort.”).¹⁴

In fact, HRSA *did not* craft the blanket exemption. As earlier observed, see *supra*, at 716, that task was undertaken by the IRS, EBSA, and CMS. See also 45 CFR § 147.132(a)(1), 147.133(a)(1) (direction by the IRS, EBSA, and CMS that HRSA’s guidelines “*must not* provide for” contraceptive coverage in the circumstances described in the blanket exemption (emphasis added)). Nowhere in 42 U. S. C. § 300gg–13(a)(4) are those agencies named, as earlier observed, see *supra*, at 717–718, an absence the Government, the Court, and the opinion concurring in the judgment do not deign to acknowledge. See Brief for HHS et al. 15–20; *ante*, at 675–679 (opinion of the Court); *ante*, at 705–706 (opinion of KAGAN, J.).

C

If the ACA does not authorize the blanket exemption, the Government urges, then the exemption granted to houses of worship in 2011 must also be invalid. Brief for HHS et al. 19–20. As the Court of Appeals explained, however, see 930 F. 3d, at 570, n. 26, the latter exemption is not attributable to the ACA’s text; it was justified on First Amend-

¹⁴ A more logical choice would have been HHS’s Office for Civil Rights (OCR), which “enforces . . . conscience and religious freedom laws” with respect to HHS programs. HHS, OCR, About Us, www.hhs.gov/ocr/about-us/index.html. Indeed, when the Senate introduced an amendment to the ACA similar in character to the blanket exemption, a measure that failed to pass, the Senate instructed that OCR administer the exemption. 158 Cong. Rec. 1415 (2012) (proposed amendment); *id.*, at 2634 (vote tabling amendment).